

HR.78 Medical & Dental Staff Annual Leave Policy

(Please also see HR.S.09 for procedural guidance)

Document Summary

This policy sets out the Trust's arrangements for the approval of annual leave for all Medical & Dental staff currently employed by Wye Valley NHS Trust and has been produced to ensure all annual leave arrangements are managed in a fair and consistent way across Wye Valley NHS Trust.

Document Number & Version	HR.78 V3 (this document is linked to HR.S.09)
Date Ratified	July 2025
Committee Ratified at	Policy Review Group
Date First Implemented	April 2014
Next Review Date	July 2030
Accountable Director	Chief Medical Officer
Document Author(s)	Medical & Dental HR Business Partner

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New Document ☐

Revised Document ☒

Applicable to ☒: Trust Wide ☒

Division: Surgical ☐ Medical ☐ Integrated Care ☐ Clinical Support ☐

Directorate / Specialty _____

Reason for producing the document	To ensure all annual leave arrangements for medical & dental staff are managed in a fair and consistent way across Wye Valley NHS Trust.
List of contributors <i>Detail designation of contributors or those consulted</i>	JLNC, Local AND Regional BMA representatives,
Review of document prior to submission <i>Detail designation or persons who reviewed and subcommittees/groups</i>	See above

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Meeting Outcome

Document for Approval: **JLNC**

Approved ☒	Committee Document Number assigned (<i>item number from Minutes</i>):	
Not approved ☐	Feedback to author:	Sent by e-mail: Click here to enter a date.

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Content checked ☒	On agenda for Meeting on: Click here to enter a date.
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Document for Ratification:

Ratified ☒	Committee Document Number assigned (<i>item number from Minutes</i>):	
	Returned to Author by e-mail on Click here to enter a date.	
Not Ratified ☐	Feedback to author:	Sent by e-mail: Click here to enter a date.

**Committee
Comments:**

For Policies only – to go to Policy Team

Final version Word document, Cover Document and minutes to be sent to Policy Team
(policy@wvt.nhs.uk)

DOCUMENT HISTORY

Catalogue and Version Number	Document Title	Date	Actioned by: (Job Title)	Page/ Section/ Paragraph	Detail Document Changes
V1	HR78	07.10.2024	Medical Workforce Manager	All	Review of document
V2	HR78	20.12.2024	Medical Workforce Manager	All	Review of document
V3	HR78	17.06.2025	Medical Workforce Manager	All	Review of document

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1. SCOPE AND PURPOSE

The aim of this policy is to provide a consistent, efficient and equitable approach to the calculation, management and application of leave for all Medical and Dental staff employed at the Trust, allowing due choice in the selection of leave, together with the effective utilisation of resources, whilst ensuring clinical safety at all times.

Operationalisation of the procedure should ensure the leave requirements of medical & dental staff are accommodated whilst meeting the needs of the service, through the job planning process, ensuring adequate levels of staff with minimal use of locum doctors and preventing disruption to patient care and achievement of Trust objectives.

The aim is for employees to take and fairly distribute their full entitlement each year other than in exceptional circumstances or where there is a contractual right. Any deliberate attempt to falsify or circumnavigate the requirements of this procedure will be investigated under Maintaining High Professional Standards (MHPS).

2. INTRODUCTION

Wye Valley NHS Trust (WVT) recognises annual leave is an important part of an employee's work-life balance and is committed to ensuring all employees can take their contractual annual leave in full during the relevant leave year period. Good management of annual leave by the employee and the manager is essential to the health and safety of the employee and patients. The right to request and take annual leave at any particular time is to be balanced against the needs of the service and patient safety.

The timing of annual leave needs to be carefully planned to ensure continuity and efficient delivery of inpatient and outpatient care and will only be authorised if there is an adequate level of staffing for the service in question. Departmental management teams must ensure there are sufficient numbers of medical & dental staff at all grades, including that of Consultant, to maintain the effective review and management of patients; minimising the necessity to rearrange both clinics and theatre lists.

This policy will be supported by the annual leave guidance for Medical & Dental staff. The guidance document will provide further detail in relation to the planning and calculation of annual leave and bank holiday arrangements.

3. DEFINITIONS

Trust employees	All medical and dental staff employed by the Trust including those managed by third-party organisations.
Manager	Line Manager/Clinical Manager with managerial responsibilities. I.e. General Managers, their deputies, Clinical Leads, Clinical Directors
Leave Coordinator	Designated person to whom leave requests are submitted
Public Holidays	English general bank/public holidays
Annual leave	Annual entitlement of leave calculated on aggregated length of service.
Leave	Combined annual and public holiday entitlement

4. DUTIES

Managers and employees must comply with their responsibilities as detailed below

4.1 People Directorate Responsibilities i.e. the Medical Workforce Team will:

- Ensure verification of employee reckonable service from the date of employment or transfer via Inter Authority Transfer (IAT). If it is not possible to verify this service, a member of the team will inform the employee who will be asked to provide documentary evidence for all periods of relevant employment, to include start and end dates. Where satisfactory evidence cannot be provided the extra allowance will not be granted. This evidence can usually be obtained from previous contracts of employment, offer letters etc.
- Confirm the leave entitlement for new employees on appointment where not automatically generated within an electronic system.
- Work with Rota Coordinators to arrange for annual leave entitlements to be amended where there is a change of contracted hours, generally following a job plan change or where reckonable service milestones have been met and ensure these changes are communicated to the member of staff.

4.2 Departmental Management team Responsibilities – i.e. General Managers, Deputy General Managers, Clinical Leads and Clinical Directors must:

- Ensure there are sufficient numbers of medical & dental staff at all grades, including that of Consultant, to maintain the effective review and management of patients; preventing pre-booked clinics being cancelled within 6 weeks of the date of the clinic and minimising the necessity to rearrange both clinics and theatre lists.
- Ensure medical & dental staff are provided with sufficient opportunities to take their annual leave entitlement within their current leave year.
- Write and publish agreed local operational arrangements on taking annual leave. These will contain rules governing the booking of annual leave in the specialty e.g. how many staff can be on leave at any one time. Templates to assist with this can be found in the Trust's SOP for annual leave and rota management.
- Be responsible for administration and management of all leave records for staff within their specialty, to ensure where possible all leave is used before the end of the leave year.
- As part of service level planning/team job planning, facilitate team discussion regarding leave preferences over peak holiday periods, to ensure equity and team working. Other meetings may also take place to discuss the leave requirements within the Specialty.
- As far as practicable ensure authorised leave does not require cover with additional hours, overtime or locum cover.
- Ensure safe and effective levels of service are maintained when leave is taken.
- Agree or reject leave within 14 calendar days of the request.
- Ensure all annual leave is appropriately recorded through relevant trust systems
- Ensure that working days with non-direct clinical care commitments form part of contracted PAs and time taken on such days must also be booked as annual leave. Non-direct clinical care commitments must never be used as a way to extend annual leave periods.

- Ensure this policy is applied fairly, consistently and equitably, whilst patient safety and service quality is maintained. With the support of the Medical Workforce Team, ensure that annual leave entitlements are amended where there is a change of contracted hours or reckonable service milestones have been met and arrange for these changes to be communicated to the member of staff.
- Monitor and discuss leave with employees to support health and well-being; ideally leave should be taken proportionately during the leave year.
- Deal with short notice leave requests sensitively and appropriately on an individual basis, ensuring it does not affect service delivery and the cost implications are minimal.
- Ensure any request for carry-over of leave is considered in accordance with section 4.2 of this policy and the use of **Appendix 3**.
- Ensure any request for the selling of leave is considered in accordance with section 5 of this policy and the use of **Appendix 4**.
- Where possible, honour any leave that may have been booked by new starters prior to their accepting employment at the Trust.
- Where possible ensure leave for medical & dental staff on a fixed term contract is taken at regular intervals within the contract period.
- Ensure employees on rotational programmes take leave proportionate to each placement.
- Take into account the needs of the service, Working Time Regulations, Health and Safety requirements and equity and fairness within the department when considering whether to authorise/reject written requests made at least 3 months in advance of 3 weeks or more in one block.

4.3 Rota teams and leave Co-ordinator responsibilities – are responsible for:

- Checking that the requested leave complies with the specialty leave parameters.
- Consulting with any other relevant parties such as other rota teams/Medical Workforce Team
- Processing requests for leave, ensuring they are approved/rejected within 14 calendar days of receipt.
- Inputting and maintaining annual leave within Trust systems, ensuring the annual leave request is in line with the agreed specialty leave parameters.
- Where applicable, checking departmental rota rules and with relevant management colleagues to ensure the leave application will not result in too many medical staff being off at any one time.
- Escalating to relevant parties where there are concerns regarding clinics, theatre lists etc. being adequately covered.
- Ensuring all employees are on Trust leave management systems and raise any queries or concerns regarding leave entitlements with the Medical Workforce Team.
- Ensuring new employees are made aware of the Leave Policy, guidance and SOP as well as local rules at induction

(The leave coordinator can be the General Manager, Clinical Lead, or a nominated individual within the service e.g. Deputy General Manager within the service/department.)

4.4 Senior Medical & Dental Staff (Consultants and SAS Doctors) - are responsible for:

- Submitting leave applications at least 8 weeks in advance of leave commencement date, best practice, (6 weeks for those areas that allow or have robust process in place), to the Head of Service/Service Director or Clinical Chair where appropriate.
- Where applicable, discussing annual leave at the annual job plan review and dates for leave and the arrangements for work to be completed during absence should be incorporated into the agreed job plan where able or alternatively agreed at least two months in advance, where possible.
- Taking steps to ensure satisfactory cover arrangements are in place and have been discussed with the Head of Service/Service Director identifying the colleague(s) who will take over full responsibility for any ongoing inpatient's care, ensuring all relevant parties are informed.
- Providing cover for colleagues subject to the terms and conditions of service, as identified above within the appropriate clinical team, i.e. a group of consultants with appropriate clinical interests.
- Contributing to the development, agreement and sharing of local leave arrangements for the specialty.
- Adhering to the agreed leave parameters for the specialty.
- Ensuring that working days with non-direct clinical care commitments form part of contracted PAs and time taken on such days must be booked as annual leave. Non-direct clinical care commitments must never be used as a way to extend annual leave periods.
- Taking their leave in either full days, ½ days, sessional hour's segments or PA's to ensure they derive the full benefits of rest, meaningful breaks from work alongside allowing for continuity of service.

4.5 All other Medical & Dental Staff – are responsible for:

- Their own awareness of the Annual Leave Policy, Guidance, local rota rules, their responsibilities and the booking procedure.
- Planning and utilising all leave entitlement within the leave year, at regular intervals throughout the leave year to facilitate a rest and break from work. Ideally proportionately during the leave year.
- Ensuring that they understand that any annual leave not taken within the relevant leave year should not be routinely carried over into the subsequent year unless there are extenuating circumstances. Where agreed, up to 5 days annual leave may be carried forward on application to the Clinical Lead. This request must be submitted using the Authorisation of Carry-over of Leave Form in **Appendix 1**.
- Complying with the agreed departmental operating procedure for booking annual leave.
- Submitting requests for leave with a minimum of six weeks (best practice eight weeks' notice).
- Participating in discussion with colleagues regarding leave requests and preferences during peak holiday periods to ensure equity and teamwork. Understand there will be popular periods in the

year e.g. July to September, Christmas, Easter, and other school holidays where it may not be possible to authorise leave due to service provision.

- Where applicable, arranging cover for leave, work with the General Manager and Clinical Lead to ensure service continuity is maintained.
- Not committing to any holiday plans until leave has been authorised.
- Submitting requests for a block of leave of 3 weeks or more in writing to the General Manager / Clinical Lead at least 3 months in advance stating the reason for the request. It is recognised that in some instances this may not be possible, therefore notice of less than 3 months will be considered on individual merit and service requirement.
- If returning late from leave, notifying the General Manager / Clinical Lead as soon as possible. Failure to do this will result in the absence being deemed unauthorised and pay will be stopped.
- Providing evidence of previous NHS service to the Medical Workforce Team if requested.
- Ensuring leave entitlements are re-calculated to reflect any changes to contracted hours. Where leave is taken that individuals are not entitled to, this could constitute fraud and will be formally investigated.
- Informing the General Manager / Clinical Lead when there is an increase in reckonable service entitlement.
- Notifying the General Manager / Clinical Lead as soon as possible where it becomes apparent it is difficult to take annual leave.
- Working days with non-direct clinical care commitments form part of contracted hours and time taken on such days must be booked as annual leave. Supporting Professional Activities or other non-direct clinical care commitments must never be used as a way to extend the annual leave period.
- Taking their leave in either full days, ½ days, sessional hour's segments or PA's to allow for continuity of service.

5. PROCEDURE DETAILS

5.1 Leave rules and General Principles

- Subject to the exigencies of the Service as described in the sections marked, carry-over of leave, and selling of annual leave below, all annual leave must be used before the end of the leave year. Leave parameters for the specialty will be agreed and must be adhered to as part of local arrangements and reviewed regularly.
- Annual leave requests must be submitted to the General Manager/Clinical Lead, following the rota rules set out for the relevant specialty, a minimum of eight weeks prior to the planned commencement of leave. Leave requests will be submitted via either **Appendix 1 or 2** until such time as an electronic leave system is in place
- The annual leave request should be approved or rejected within a period of 14 calendar days.
- Doctor and Dentists who are not in the Trust when they are scheduled for SPA, flexible PA or DDC admin should book this time accordingly either as annual leave or study leave as they still form part of contracted hours. Even when SPA sessions are worked at home or at another site

(these should be clearly marked in the job plan). If the individual is not able to return to the Trust should a clinical need were to arise, then it should always be booked as leave.

- Leave should be planned throughout the year to enable a regular rest from work.
- Requests to take three or more than three consecutive weeks will be subjected to review by the relevant General Manager/Clinical Lead who will consider the service requirements, equity and fairness within the department before deciding whether to authorise or reject the request for a period of extended leave.

5.2 Carry Over of Leave

- All leave must be taken within the leave year; any annual leave not taken within the relevant leave year may not be routinely carried over into the subsequent year. However, subject to the exigencies of the service, up to 5 days annual leave may be carried over. Pro-rata for part-time staff.
- Application for carry-over of leave must be formally submitted on the form give in **Appendix 3**. Carry-over will be agreed provided the carry-over is requested due to exigencies of the service. If carry-over is not applied for prior to the end of the leave year, it will be forfeited.
- General Managers should review DCC delivery as carry over may relate to non-direct clinical care activities. This may be carried over but should not reduce direct clinical care delivery going forward
- General Managers should review any WLI undertaken on planned leave as this leave should not be carried forward due to the additional payment
- General Managers / Clinical Leads must ensure authorised carried over leave is recorded on Trust systems, and used before the annual leave entitlement for the next year.
- Leave is accrued during maternity, adoption and paternity/partner leave. For further information see the Maternity, paternity & Adoption Leave Policy on how carry over applies in these instances (HR.62).

5.3 Selling annual leave

The Trust would prefer staff to use their annual leave entitlements within the receiving leave year as far as possible. However staff may apply to sell a proportion of their annual leave entitlement during the leave year. This will be a maximum of 5 days and will be pro-rata for part time staff.

Once the decision has been made to convert annual leave into pay it cannot be revoked for the current leave year. The decision does not affect future years' entitlements.

All annual leave sold will be paid at the basic rate of pay. The amount to be paid will be paid as a lump sum in the next available pay month. Any member of staff wishing to apply should complete the form given at **Appendix 4** and submit to their Clinical Director for consideration.

Please note that the Trust wish staff to take leave entitlement to support a good work life balance – therefore the application to sell annual leave must be supported by exceptional circumstances.

5.4 Sickness Absence and Leave

- If an employee falls sick whilst on Annual Leave they must follow the sickness notification process in the Trust's Managing Sickness Absence Policy (HR.13). A self-certificate may cover days 1 to

7 of the period of sickness, however a medical fit note is required for subsequent days. A fit note is required prior to the leave balance being adjusted.

- If an employee who is absent due to sickness has pre-booked annual leave and plans to go away, this must be discussed with the General Manager/Clinical Lead and will be counted as annual leave, as the employee is unavailable to meet with the employer.
- Employees will not be entitled to an additional day off if sick on a bank/public holiday that they would otherwise have been required to work as part of their normal working week.

5.5 Short Notice Leave

- Subject to suitable alternative arrangements having been made, Consultants may take up to two days of their annual leave without seeking formal permission, provided that they give notification beforehand (Schedule 18, Section A, Terms and Conditions Consultants (England) 2003).

5.6 Leaving the Trust

- Employees who leave the Trust will receive 1/12th of their leave entitlement for each completed calendar month worked in the current leave year, less any leave taken.
- Employees will normally be allowed to use outstanding leave during their notice period. However, the Trust reserves the right to refuse these requests where service delivery will be compromised.
- Payment in lieu of outstanding leave will only be made in exceptional circumstances i.e. where it has not been possible for the employee to take remaining leave due to the requirements of the service.
- Where leave taken exceeds the leave entitlement, a deduction will be made from the employee's final salary payment.

5.7 Leave during periods of exclusion

- An individual excluded within MHPS may take annual leave but should first seek agreement from their General Manager/Clinical Lead via the nominated point of contact.

6. PUBLIC HOLIDAYS

A Public Holiday is defined as a period of 24 hours from midnight to midnight.

The Trust recognises the following Public Holidays:

Christmas Day	Boxing Day	New Year's Day	Good Friday
Easter Monday	Spring Public Holiday	May Day	Late Summer Holiday

Only those Bank Holidays which fall during the period of employment should be added to the leave entitlement.

7. LEAVE ENTITLEMENTS

All the leave entitlements are based on full time (40 hours for Medical staff and 37 ½ hours for Dental staff) contracts. Part time staff are entitled to the same annual leave entitlement on a pro-rata basis.

Leave entitlements are in line with the agreed National Terms and Conditions of Service. For Medical Staff other than Resident Doctors, the leave year runs from the anniversary date of the staff members appointment or adjusted to a common start date in force in that employment. No detriment to the staff member will arise from the leave year adjustment.

The current leave entitlements are as follows:

7.1 Resident Doctors Annual Leave Entitlements

The following practitioners shall be entitled to annual leave with full pay at the rate of 5 weeks plus 2 statutory days a year. 216 hours / 27 days	
Foundation Year 1 and 2 Resident Doctors at 1st or 2nd incremental points on their pay scale (years 1-3)	
Core Trainees/Specialty Trainees at the 1st or 2nd incremental points on their pay scale (years 1-3)	
The following practitioners shall be entitled to annual leave with full pay at the rate of 6 weeks plus 2 statutory days a year or 256 hours / 64 PA's :	
Core Trainees/Specialty Trainees on their 3rd incremental point on the pay scale (yr. 4 onwards)	

The leave year for resident doctors will commence as per their start date in training post i.e. August each year and will be calculated on a pro rata basis. For details on the specific dates, please see the contract of employment. The entitlement to public holidays is in addition to the annual leave entitlement.

Resident doctors are asked to proportion their leave equally throughout the leave year where they rotate between Specialties as part of their training programme. All annual leave for the time within a given specialty must be taken prior to rotating out of that specialty. Should this not be possible due to extenuating circumstances, this should be discussed with the General Manager/Clinical Lead for the specialty that the resident doctor is rotating to and a minimum of eight weeks before the individual rotates into that department, due to the national code of practice required for issuing rotas.

Resident doctors must submit their leave requests as per this local rota rules and trust annual leave guidance, these requests will then be reviewed by the department leave coordinator and authorised or rejected within 14 calendar days.

7.2 Specialists / Associate Specialists and Specialty Doctors Annual Leave Entitlements

SAS doctors are entitled to Annual Leave at the following rates per year and public holiday entitlement is in addition to this. Further information in relation to bank holidays can be found in the annual leave guidance document for Medical & Dental staff.

The leave year either runs from the anniversary date of the SAS doctor's appointment or it may be adjusted to a common start date. Part time SAS doctors are entitled to the same annual leave as a whole time equivalent on a pro rata basis.

Grade	Entitlement		
Associate Specialist – old contract pre 2008 and 2008 contracts	Days	Hours	PA's
7 years or less completed service in the grade	32	256	64
7 years or more completed service in the grade	34 *	272	68
Grade	Entitlement		
Specialty Doctor (2008)			
Up to 2 years completed service in the grade	27	216	54
2 – 7 years' service in the grade	32	256	64
7 years or more completed service in the grade	34 *	272	68
SAS (Specialty Doctor & Specialist on the new contract from 1st April 2021)			
Up to 2 years completed service in the grade where not previously worked in the Specialty Doctor grade or equivalent	27	216	54
Over 2 years' service in the grade	32	256	64

*** Denotes the additional days provided above contractual entitlements**

7.3 Dentist Annual leave Entitlements

Grade	Entitlement days/hours
Dentists on the first two salary points of band A	27 days per annum / 216 hours
All other dentists on the scale	32 days per annum * / 256 hours

The leave year runs from the anniversary date of the Dentist's appointment, or it may be adjusted to a common start date. No detriment to the Dentist will arise from the leave year adjustment.

Part time Dentists are entitled to the same annual leave as a whole time equivalent on a pro rata basis.

The entitlement to public holidays is in addition to the annual leave entitlement. Further information in relation to bank holidays can be found in the annual leave guidance document for Medical & Dental staff.

7.4 Consultants Annual Leave Entitlements

Consultants are entitled to Annual Leave at the following rates per year, exclusive of public holidays but inclusive of extra statutory days:

Grade	Entitlement		
Consultant - based on service at this grade	Days	Hours	PA's

Appointed up until the 31 st March 2004 + 2 statutory days and 2 locally agreed days	* 34	272	68
Up to 7 years' service and appointed between 1 st April 2004 and 31 st March 2005 + 2 statutory days and 2 locally agreed days	* 34	272	68
With 7 years + service and appointed between 1 st April 2004 and 31 st March 2005 + 2 statutory days and 2 locally agreed days	*35	280	70
Grade	Entitlement		
Up to 7 years' service and appointed from the 1 st April 2005 + 2 statutory days and 2 locally agreed days	*34	272	68
With 7 years + service and appointed after the 1st April 2005 + 2 statutory days and 2 locally agreed days	*36	288	72

Entitlement shown include the two extra statutory days previously available

*** denotes the additional days provided above contractual entitlements.**

The leave year runs from the anniversary date of the Consultant's appointment or it may be adjusted to a common start date. No detriment to the Consultant will arise from the leave year adjustment. Part time Consultants are entitled to the same annual leave as a whole time equivalent on a pro rata basis.

The entitlement to public holidays that fall within the leave year are in addition to the annual leave entitlement. Further information in relation to bank holidays can be found in the annual leave guidance document for Medical & Dental staff.

7.5 For the purpose of calculating leave in Weeks or Days

Normal working week

A week, for the purposes of annual leave entitlement, consists of whatever constitutes the member of staff's normal working week e.g. if a consultant works a normal working week 40 hours / 5 days, this is the number of hours/days required to consume a weeks' worth of leave. Alternatively if the job plan consists of a fixed day off working 40 hours / 4 days a week, this is the number of days required to consume a weeks' worth of leave.

Normal working day

Will be calculated on the average daily job planned activity. i.e. where a practitioner is full time (10 PA's or more) and working 5 days, they will consume 1/5th of their planned leave entitlement for every week of leave accessed.

A full time practitioner working 4 days, will need to consume 1.25 days of their planned leave for every working day to ensure they consume the relevant daily leave entitlement to cover a weeks' worth of leave.

Normal working day – Public Holidays

For the calculation of a public holiday, 8 hours is used as standard for a normal working day calculation, this is calculated as 1/5th of the standard working week (40 hours over 5 days).

If you are scheduled to work on a public holiday, the entitlement accrued will remain in your leave pot for you to use at an alternative time. If you are not required to work on a public holiday the entitlement being retained to be used at an alternative time, up to the value accrued for the public holiday.

8. CONSULTATION AND APPROVAL

Consultation of this procedure has taken place through the Joint Local Negotiating Committee and the Hospital Medical Committee.

Authorisation of leave will be made as follows:-

Employee	Authoriser
Chief Medical Officer	Chief Executive/Head of Service/Service Director
Associate Chief Medical Officers/Deputy Chief Medical Officers	Clinical Directors in their Specialty
Clinical Director	Associate Chief Medical Officer
Clinical Lead	Clinical Director OR Associate Chief Medical Officer
Consultants	Clinical Director/Clinical Lead
Medical Staff	Clinical Lead but can be delegated to Educational Supervisor for Resident doctors dependent on Divisional structure

9. TRAINING

Medical & Dental staff will be made aware of the Annual leave procedure that contractually applies to them and the associated guidance on commencement in post. Any procedural changes will be agreed with the JLNC and communicated to relevant management teams and all Medical & Dental staff.

Any training on systems used as part of the leave management process will be given to practitioners as they join the Trust. Ongoing support will be provided to all relevant departmental and divisional management teams with the management of annual leave.

10. MONITORING COMPLIANCE WITH THIS DOCUMENT

The table below outlines the Trust's monitoring arrangements for this document.

Registered Audit Reference Number: [Insert reference number here]					
Aspect of compliance or effectiveness being monitored	Monitoring Method	Individual responsible for the monitoring	Frequency of the monitoring activity	Group/ committee which will receive the findings / monitoring report	Group / committee / individual responsible for ensuring that the actions are completed

Leave request and authorisation compliance	Audit	Departmental management teams	6 monthly	JLNC	
Leave against Rota compliance	Audit	Medical Workforce team	annually	Job Planning Committee	

11. RELATED TRUST DOCUMENTS: POLICIES / PROCEDURES / GUIDELINES

Leave guidance for Medical Staff

12. REFERENCES

<http://nhsemployers.org>

13. EQUALITY IMPACT ASSESSMENT

The Equality Impact Assessment has been completed. The assessment document is held centrally and is available by contacting policies@wvt.nhs.uk.

14. Appendix 1: Example leave request sheet for Resident Doctors

NAME	Full name here					POST TITLE	Enter post position title here
A/L YEAR DATES	Enter start date and end date here					A/L ENTITLEMENT	
NOTES:	Enter LTFT percentage here					BANK HOLIDAYS	8
						TOTAL	8
ENTITLEMENT USAGE						Bank Holidays	
Annual Leave Remaining	From	To	DAYS TAKEN	Approved/Declined	Authorised by and date	BH Dates	Taken in Lieu
0	First Rotation??/??/?? - ??/??/?? = Recommended 9 Days					??	
	Second Rotation??/??/?? - ??/??/?? = Recommended 9 Days					??	
						??	
						??	
	Third Rotation??/??/?? - ??/??/?? = Recommended 9 Days					??	
						??	
						??	
						??	
	Total Days Taken:		0			-	

15. Appendix 2

Example Annual Leave Request Form

Consultants, Associate Specialists and Specialty Doctors

Applicant

Name:		Specialty:	
Grade:		Directorate:	

Leave from (date):	Leave to (date):	Leave for this request (Days/PA's/Hours):	Leave remaining (Days):

Where applicable, please give the name of the colleague(s) who has agreed to cover your duties:

Name of Covering Colleague(s):	Duties being covered:

Does your leave mean that service changes may be required? Please detail the date and time and which services are affected (e.g. Outpatients, Theatres, Diagnostics, Other Departments etc.):

--

Signature of applicant:..... Date:.....

Please ensure this form reaches the relevant personnel in good time for approval to be given at least 6 (six) weeks prior to the leave requested

Authorising manager

I do / do not (please delete as appropriate) approve this annual leave request. If the request is refused, the reasons are as follows. The rota has been checked and approval of this leave would mean:

☐ Inadequate notice has been given
 ☐ inadequate staffing levels to maintain services

Other:

--

Signature:..... Date:.....

Please process this annual leave request and return to the applicant within 2 (two) weeks of receipt

16. Appendix 3

Application for the carry-over of Annual leave
(Medical & Dental staff only)

Applicant

Name:		Specialty:	
Grade:		Directorate:	

Yearly Annual leave entitlement:

Amount of A/L to be carried over (in Days/Hours/PA's)	Employee to state reason for carry over request (signed & dated)	General Manager/Clinical Lead to confirm/reject reason for carry over (sign & date)	Clinical Director to approve or reject carry over request for all staff groups if more than 5 days carry over requested (sign & date)
	Signed:	Signed:	Signed:
	Print name:	Print name:	Print name:
	Date:	Date:	Date:

17. Appendix 4

REQUEST TO SELL ANNUAL LEAVE FORM

Applicant

Name:		Specialty:	
Grade:		Directorate:	
Leave entitlement in days			

For the leave year ___/___ I wish to selldays/hours/PA’s annual leave.

I understand that approval of the sale of my annual leave is at the discretion of the Clinical Director and in accordance with Trust policy. I understand that I will forego these days from my annual leave entitlement and that payment will be made at my basic daily rate of pay (excluding payments for additional activity or enhancements).

This will be paid to me in a lump sum (normally in the next available pay month) which will not be pensionable but will attract tax and national insurance contributions. I understand that should I leave part way through the leave year a re-calculation of my annual leave will be undertaken.

If I have sold more than my entitlement, I will be required to repay any over payment from my salary.

Signed..... Date

APPROVAL

Name of Clinical Director approving request:

Signature of Clinical Director

Date:.....

PLEASE SEND THE COMPLETED FORM, WITH CLINICAL DIRECTOR SIGNATURE, TO THE TRUSTS PAYROLL DEPARTMENT & THE MEDICAL & DENTAL WORKFORCE MANAGER, RESOURCING TEAM, HR